

Dermatology: Normal Skin, Anatomy, and Lesion Morphology

Board-Review Quality • Comprehensive Academic Reference

- Dermatology: Normal Skin, Anatomy, and Lesion Morphology
 - 1 Chapters • Premium Tables • Key Teaching Points
 - OSCE Pearls • MCQ Traps • High-Yield Facts

2025 Edition

TABLE OF CONTENTS

Chapter 1 Dermatology: Normal Skin, Anatomy, and Lesion Morphology

Dermatology: Normal Skin, Anatomy, and Lesion Morphology

Overview: The Skin as a Window to Health

The skin is far more than a superficial wrapping for the human body — it is the largest organ, accounting for approximately 15% of total body weight and covering a surface area of roughly 1.5 to 2 square meters in the average adult. It serves as the body's first line of defense against the external environment and plays essential roles in thermoregulation, sensation, immune surveillance, and fluid homeostasis. Critically for the clinician, the skin can harbor important clues to underlying systemic disease, nutritional deficiency, malignancy, infection, and autoimmune conditions. A thorough, systematic approach to skin examination — encompassing the entire cutaneous surface, nails, hair, and mucous membranes — is therefore an indispensable component of every comprehensive clinical assessment.

Beyond its physical manifestations, dermatologic disease carries a significant psychosocial burden. Conditions such as psoriasis, acne, eczema, and vitiligo can profoundly impair self-esteem, social functioning, and mental health. The **Dermatology Life Quality Index (DLQI)** is a validated, widely used, 10-item questionnaire that quantifies the impact of skin disease on a patient's quality of life over the preceding week. Scores range from 0 (no impact) to 30 (extremely large impact), making it a practical tool for both clinical decision-making and research.

KEY TEACHING POINTS

The skin is the **largest organ of the body** and serves as a living biological barrier, thermoregulator, and preventer of transepidermal water loss.

Cutaneous changes are frequently the **first sign of systemic disease** — the skin is a mirror of internal pathology.

A complete dermatologic examination must include the **entire skin surface, nails, hair, and mucous membranes**.

The **Dermatology Life Quality Index (DLQI)** is a validated, 10-question tool (score 0–30) that measures the psychosocial burden of skin disease.

MCQ TRAPS & PITFALLS

Do NOT forget to examine nails, hair, and mucous membranes — focal findings in these areas may be the only clue to a systemic condition.

A skin lesion that fails to respond to standard topical therapy should prompt consideration of an underlying systemic cause.

Skin Anatomy: The Three Layers

Understanding skin anatomy is fundamental to comprehending dermatologic pathology, interpreting clinical signs, and making accurate diagnoses. The skin is composed of three principal layers, each with distinct structures and functions.

1. Epidermis

The epidermis is the outermost layer of the skin and is composed primarily of **keratinocytes** — specialized squamous epithelial cells that undergo a process of terminal differentiation (keratinization) as they migrate from the basal layer to the surface. The epidermis is **avascular**, receiving its nutrients via diffusion from the underlying dermal capillaries. It is rich in **keratin**, a structural protein responsible for mechanical strength and waterproofing.

The epidermis itself is further subdivided into five strata (from deep to superficial):

- **Stratum basale (germinativum):** The deepest layer where active mitosis occurs; contains melanocytes and Merkel cells.
- **Stratum spinosum:** Characterized by desmosomal connections between keratinocytes, giving a "prickle cell" appearance on histology.
- **Stratum granulosum:** Keratohyalin granules appear; the transition point where cells begin to lose their nuclei.
- **Stratum lucidum:** A thin, translucent layer found only in thick skin (palms and soles).
- **Stratum corneum:** The outermost layer of terminally differentiated, anucleate corneocytes; provides the primary barrier function.

Other important cells residing in the epidermis include **melanocytes** (produce melanin), **Langerhans cells** (antigen-presenting dendritic cells involved in immune surveillance), and **Merkel cells** (mechanoreceptors).

Epidermal Layers (Deep to Superficial)

Layer	Key Features	Key Cells
Stratum basale	Deepest layer; active mitosis	Melanocytes, Merkel cells, keratinocyte stem cells
Stratum spinosum	Desmosomal connections ("prickle cells")	Keratinocytes, Langerhans cells
Stratum granulosum	Keratohyalin granules; nuclear loss begins	Keratinocytes
Stratum lucidum	Thin, translucent; thick skin only (palms/soles)	Anucleate keratinocytes
Stratum corneum	Outermost barrier; anucleate corneocytes	Terminally differentiated corneocytes

2. Dermis

The dermis lies beneath the epidermis and is separated from it by the **basement membrane zone**. It is a connective tissue layer that provides structural support, elasticity, and nourishment to the skin. The dermis is subdivided into the **papillary dermis** (superficial, thin, with loose connective tissue and dermal papillae that interdigitate with the epidermis) and the **reticular dermis** (deeper, thicker, with dense irregular connective tissue).

Key structures within the dermis include:

- **Connective tissue:** Collagen fibers (types I and III) provide tensile strength; elastin fibers provide elasticity.
- **Blood vessels:** A superficial and deep vascular plexus supplies nutrients and mediates thermoregulation.
- **Lymphatic vessels:** Drain interstitial fluid and transport immune cells.

- **Nerves:** Both sensory (Meissner corpuscles for light touch, Pacinian corpuscles for deep pressure and vibration, free nerve endings for pain and temperature) and autonomic (innervating blood vessels, arrector pili muscles, and sweat glands).
- **Hair follicles:** Complex mini-organs extending from the epidermis into the dermis and sometimes subcutis.
- **Sweat glands:** Eccrine glands (distributed widely, secrete hypotonic sweat for thermoregulation) and apocrine glands (limited to axillae, groin, and areolae, produce viscous secretion).
- **Sebaceous glands:** Produce sebum; associated with hair follicles.

3. Subcutaneous Fat (Hypodermis/Subcutis)

The subcutaneous layer, also known as the **hypodermis** or **subcutis**, lies beneath the dermis and is composed predominantly of **adipose tissue** (fat lobules separated by fibrous septa). This layer provides insulation, energy storage, mechanical cushioning, and anchorage to underlying structures such as fascia and muscle.

KEY TEACHING POINTS

The **epidermis is avascular** — it receives nutrients by diffusion from dermal capillaries.

The **stratum basale** is where mitosis occurs; the **stratum corneum** is the barrier layer.

The dermis contains **blood vessels, lymphatics, nerves, hair follicles, and sweat glands** — damage to the dermis can affect all of these structures.

Eccrine sweat glands are distributed widely over the body surface, while **apocrine glands** are confined to axillae, groin, and areolae.

The **subcutis** provides insulation, cushioning, and energy storage.

Skin Lesion Morphology: Primary and Secondary Lesions

A **skin lesion** refers to any area of skin that differs in color, texture, or structure from the surrounding normal skin. The accurate description and classification of skin lesions using standardized terminology is the foundation of dermatologic diagnosis and essential for effective communication between clinicians.

Skin lesions are divided into **primary** and **secondary** types.

- **Primary skin lesions** originate on previously **healthy skin** and are directly associated with a specific disease process.
- **Secondary skin lesions** develop from the **evolution** of a primary lesion due to traumatic manipulation (scratching, rubbing), treatment, or natural disease progression.

OSCE PEARLS

Always describe skin lesions using precise morphologic terminology — this is the language of dermatology.

A systematic approach: note the **type** of lesion, **size**, **color**, **shape**, **distribution**, **arrangement**, and **border** (whether well- or ill-defined).

Use a dermatoscope when encountering pigmented lesions — it improves diagnostic accuracy for melanoma.

Primary Skin Lesions

Macules and Patches

Macules and patches are **flat, non-palpable** lesions characterized by a change in skin color without any elevation or depression above or below the surface of the surrounding skin. The color change may be erythematous (red), hypopigmented (lighter), or hyperpigmented (darker) compared with adjacent skin.

- **Macule:** A flat lesion measuring **<1 cm** in diameter. *Example:* a freckle (ephelis).
- **Patch:** A flat lesion measuring **>1 cm** in diameter. *Example:* a patch of vitiligo.

KEY TEACHING POINTS

Macules and patches are distinguished **solely by size** — both are flat, non-palpable, and differ from surrounding skin only by color.

Macules do **not blanch** when due to hemorrhage (e.g., petechiae); they **do blanch** when due to vascular dilation.

Macules vs Patches

Feature	Macule	Patch
Size	<1 cm	>1 cm
Elevation	None (flat)	None (flat)
Color change	Yes (erythema, hypo-/hyperpigmentation)	Yes
Examples	Freckle, junctional nevus, petechiae	Vitiligo, port-wine stain, Mongolian spot

Papules and Plaques

Papules and plaques are **raised, palpable** solid lesions.

- **Papule:** A raised lesion measuring **<1 cm** in diameter. *Example:* a mole (dermal nevus), insect bite.
- **Plaque:** A raised, flat-topped lesion measuring **>1 cm** in diameter, often formed by the confluence of multiple papules. *Example:* psoriasis plaque, lichen planus plaque.

Papules vs Plaques

Feature	Papule	Plaque
Size	<1 cm	>1 cm
Elevation	Raised, palpable	Raised, flat-topped
Composition	Solid (no fluid)	Solid (may be confluent papules)

Feature	Papule	Plaque
Examples	Mole, insect bite, seborrheic keratosis	Psoriasis, lichen planus, mycosis fungoides

Nodules

A **nodule** is a **palpable, circumscribed, solid lesion** with a volume greater than that of a papule. Nodules are **>1 cm in diameter** and involve the **dermis and/or subcutaneous fat** (subcutis).

- **Example:** dermatofibroma, lipoma, nodular melanoma, epidermoid cyst, rheumatoid nodule.

KEY TEACHING POINTS

Nodules are **deeper and larger** than papules; they extend into the dermis or subcutis.

Any new, growing, or changing nodule warrants biopsy to exclude malignancy.

Vesicles and Bullae

Vesicles and bullae are **fluid-filled lesions (blisters)**.

- **Vesicle:** A blister measuring **<1 cm** in diameter. *Example:* herpes simplex, chickenpox (varicella), contact dermatitis.
- **Bulla (plural: bullae):** A blister measuring **>1 cm** in diameter. *Example:* bullous pemphigoid, bullous impetigo, burn blisters.

MCQ TRAPS & PITFALLS

Do NOT confuse vesicles (small blisters) with pustules (pus-filled lesions) — the content, not just the morphology, determines the correct term.

Bullous pemphigoid typically presents with large, tense bullae, while **pemphigus vulgaris** presents with flaccid bullae that rupture easily.

Vesicles vs Bullae

Feature	Vesicle	Bulla
Size	<1 cm	>1 cm
Content	Clear fluid	Clear, serous, or hemorrhagic fluid
Location	Intraepidermal or subepidermal	Intraepidermal or subepidermal
Examples	Herpes simplex, chickenpox, dermatitis herpetiformis	Bullous pemphigoid, bullous impetigo, burns

Pustules

A **pustule** is a **pus-filled lesion**. Pustules may arise from either infectious or sterile (non-infectious) processes.

- **Infectious examples:** Staphylococcal folliculitis, impetigo.
- **Sterile examples:** Pustular psoriasis, pustular drug eruption (acute generalized exanthematous pustulosis — AGEP).

KEY TEACHING POINTS

Pustules contain **purulent material** (neutrophils, bacteria, or debris) — this distinguishes them from vesicles, which contain clear fluid.

Sterile pustules are important — assume infection only when clinical and microbiological evidence supports it.

Wheals

A **wheal** is a **transient elevation of the skin** caused by dermal edema. Wheals are typically:

- Pale centrally with an erythematous (red) rim
- Pruritic (itchy)
- Evanescent — they appear and disappear rapidly, often lasting <24 hours in any one location

Wheals are the hallmark lesion of **urticaria (hives)** and are mediated by mast cell degranulation and histamine release.

OSCE PEARLS

The **transient** nature of wheals (lasting <24 hours in a single location) is a key distinguishing feature from other papular or plaque-forming conditions.

If "urticarial" lesions persist >24 hours in the same location, consider **urticarial vasculitis** rather than simple urticaria.

Compound and Secondary Skin Lesions

Maculopapular Rash

A **maculopapular rash** is a clinical term describing a **collection of mixed skin lesions** — some flat (macules) and some raised (papules) — distributed over the skin surface. It is not a single morphologic entity but rather a pattern of eruption.

Common causes include:

- **Drug eruptions** (e.g., amoxicillin rash in mononucleosis, sulfonamides, anticonvulsants)
- **Viral exanthems** (e.g., measles, rubella, erythema infectiosum, roseola)
- **Bacterial exanthems** (e.g., scarlet fever caused by *Streptococcus pyogenes* producing erythrogenic toxin)
- **Spirochetal infections** (e.g., secondary syphilis — maculopapular rash involving palms and soles is classic)

KEY TEACHING POINTS

A maculopapular rash is a **pattern**, not a specific diagnosis — the history, distribution, and associated features point to the etiology.

In secondary syphilis, a maculopapular rash involving the **palms and soles** is a critical clue — always examine these sites.

Lichenification

Lichenification refers to **hard, thickened skin with accentuated skin markings** (the normal lines and furrows of the skin become exaggerated). It results from **chronic inflammation combined with repetitive rubbing or scratching** most commonly seen in chronic eczema (atopic dermatitis), lichen simplex chronicus, and other pruritic conditions.

Lichenification represents the skin's response to chronic mechanical trauma. While it begins as pruritus leading to scratching, over time the thickened skin itself becomes more itchy, creating an **itch-scratch cycle**.

Lichenification vs Related Features

Feature	Lichenification	Lichen Planus	Lichen Sclerosus
Cause	Chronic rubbing/scratching	Autoimmune (T-cell mediated)	Autoimmune; chronic inflammatory
Appearance	Thickened, accentuated skin lines	Purple, polygonal, pruritic papules	White, atrophic plaques; may cause scarring
Common sites	Neck, ankles, wrists, genitalia	Wrists, ankles, oral mucosa	Genitalia (vulva, penis)

Scale

Scale refers to **hyperkeratosis** — the accumulation of excess stratum corneum due to either increased keratinocyte proliferation or delayed desquamation (shedding of dead skin cells). Scale appears as visible peeling or flaking of the outermost skin layer.

- **Examples:** Psoriasis (silvery-white scale), lichen planus (fine, lacy white scale — Wickham striae), eczema (fine, dry scale), pityriasis rosea (collarette scale), tinea infections, ichthyosis.

KEY TEACHING POINTS

Scale indicates a **disorder of keratinization** — too much production or too little shedding of the stratum corneum.

In psoriasis, scale is **silvery-white** and adherent; attempting to remove it reveals pinpoint bleeding (**Auspitz sign**).

Wickham striae (fine, lacy, white scale on papules) is pathognomonic of lichen planus.

Crust

Crust refers to **dried serum, blood, or pus** on the surface of the skin. It forms when the fluid content of a vesicle, bulla, pustule, or excoriation dries and hardens on the skin surface.

- **Example:** impetigo (golden-yellow crust), herpes simplex, eczema with secondary infection.

MCQ TRAPS & PITFALLS

Crust is a **secondary lesion** — it results from the evolution or manipulation of a primary blister, pustule, or erosion.

In impetigo, the characteristic **golden-yellow (honey-colored) crust** is a key diagnostic feature.

Fissure

A **fissure** is a **linear crack or slit extending through the full thickness of the epidermis and into the dermis**. Fissures are typically **painful** due to exposure of dermal nerve endings.

- **Examples:** angular cheilitis (fissures at the corners of the mouth), hand dermatitis (fissures on the fingers and palms), heel fissures.

Erosion

An **erosion** is a **partial-thickness loss of the epidermis** that heals **without scarring**. Because the basement membrane zone and dermis are preserved, re-epithelialization occurs from the surrounding intact epidermis and skin appendages (hair follicles, sweat glands).

- **Examples:** impetigo, friction blisters after rupture, superficial burns, pemphigus vulgaris (fragile blisters easily rupture, leaving erosions).

KEY TEACHING POINTS

Erosions heal **without scarring**; ulcers heal **with scarring** — this is the key distinguishing feature.

In pemphigus vulgaris, the Nikolsky sign (sliding pressure causing epidermal detachment) reveals the fragility of the epithelium.

Erosion vs Ulcer

Feature	Erosion	Ulcer
Depth	Partial (epidermis only)	Full-thickness (epidermis + upper dermis)
Scarring (none)	Heals without scar	Heals with scar
Blanchable base	Yes	No (granulation tissue visible)
Examples	Ruptured blister, impetigo, pemphigus	Stasis ulcer, pyoderma gangrenosum, pressure ulcer

Ulcer

An **ulcer** is a **full-thickness loss of the epidermis and upper dermis**. Because the dermis is destroyed, healing occurs by granulation tissue formation and wound contraction, resulting in **scarring**.

- **Examples:** venous stasis ulcer, arterial ulcer, pyoderma gangrenosum, pressure ulcer, neuropathic ulcer (diabetic foot ulcer).

OSCE PEARLS

When you see **pyoderma gangrenosum**, remember that it is a **diagnosis of exclusion** and shows **pathergy** (worsening with surgical debridement) — do NOT debride these ulcers.

Excoriations

Excoriations are **partial or complete loss of the epidermis caused by scratching**. They represent self-inflicted trauma from the patient's attempt to relieve pruritus.

- **Examples:** neurotic excoriations (psychogenic excoriation disorder), acne excoriée (compulsive picking of acne lesions), prurigo nodularis.

KEY TEACHING POINTS

Excoriations are iatrogenic in the sense that they are self-inflicted — always consider the underlying cause of pruritus.

In patients with unexplained, linear, or geometric erosions, factitious dermatitis (dermatitis artefacta) should be considered.

Atrophy

Atrophy refers to **loss of tissue** affecting the epidermis, dermis, or subcutaneous fat. Thinning of the epidermis is characterized by:

- Loss of normal skin markings (wrinkles appear)
- Loss of pigment (hypopigmentation)
- Translucent appearance (underlying structures such as blood vessels become visible)
- Sclerosis (hardening) of the underlying connective tissue
- Telangiectasia (dilated superficial blood vessels)
- Evidence of diminished blood supply
- **Examples:** lichen sclerosus, skin aging (senile atrophy), poikiloderma, striae (stretch marks), corticosteroid side effects (topical or systemic).

Atrophic Skin Changes — Causes and Features

Cause	Epidermal/Dermal/Fat	Key Features
Aging (senile atrophy)	Epidermal + dermal	Thin, wrinkled, easy bruising
Corticosteroid use	Dermal	Fragile skin, easy bruising, striae, telangiectasia
Lichen sclerosus	Epidermal	White, parchment-like plaques; scarring
Striae (stretch marks)	Dermal	Pink → white lines; collagen disruption
Poikiloderma	Epidermal + dermal	Atrophy + telangiectasia + pigmentary changes

Desquamation

Desquamation refers to the **peeling or flaking of superficial scales** of the stratum corneum. It represents the normal physiologic shedding of dead skin cells, which may become clinically visible when the process is accelerated or the scale becomes confluent.

- **Examples:** post-streptococcal desquamation (peeling of fingertips after scarlet fever), toxic shock syndrome desquamation, post-inflammatory peeling in Kawasaki disease.

Quick Reference: Primary vs Secondary Lesions Summary

Classification of Skin Lesions

Category	Lesion Type	Key Feature	Size Threshold
Primary	Macule	Flat, color change only	<1 cm
Primary	Patch	Flat, color change only	>1 cm
Primary	Papule	Raised, solid	<1 cm
Primary	Plaque	Raised, solid, flat-topped	>1 cm
Primary	Nodule	Raised, solid, deep (dermis/subcutis)	>1 cm
Primary	Vesicle	Fluid-filled (clear)	<1 cm
Primary	Bulla	Fluid-filled (clear/serous)	>1 cm
Primary	Pustule	Pus-filled	Any size
Secondary	Scale	Accumulated stratum corneum	—
Secondary	Crust	Dried serum/blood/pus	—
Secondary	Fissure	Linear crack through epidermis	—
Secondary	Erosion	Partial epidermal loss; heals without scar	—
Secondary	Ulcer	Full-thickness loss; heals with scar	—
Secondary	Excoriation	Epidermal loss from scratching	—
Secondary	Lichenification	Thickened skin with accentuated markings	—
Secondary	Atrophy	Loss of tissue; thinning	—
Secondary	Desquamation	Peeling of superficial scales	—

Category	Lesion Type	Key Feature	Size Threshold
Compound	Maculopapular rash	Mixed macules and papules	—
Compound	Wheal	Transient, edematous, itchy elevation	—

Practice Questions

Q: A 25-year-old woman presents with multiple well-demarcated, flat, depigmented patches on her face and hands. The patches are non-scaly and have been slowly enlarging over the past year. Which of the following is the most likely primary skin lesion described?

- A. Plaque
- B. Nodule
- C. Patch
- D. Papule
- E. Macule

Answer: C

*Explanation: The key features are **flat** (non-raised), **color change** (depigmented/hypopigmented), and **size >1 cm** (described as "patches" on face and hands). By definition, a flat lesion >1 cm is a **patch**. A macule is the same but <1 cm. Plaques are raised, nodules are deep and solid, and papules are raised and <1 cm.*

Q: A 6-year-old child presents with an itchy vesicular rash on the trunk and face. Some lesions have ruptured, leaving superficial erosions that heal without scarring. The most likely diagnosis is:

- A. Bullous pemphigoid
- B. Pemphigus vulgaris
- C. Chickenpox (varicella)
- D. Bullous impetigo
- E. Epidermolysis bullosa

Answer: C

*Explanation: The combination of **vesicles** (fluid-filled, <1 cm lesions), **trunk and face distribution**, **pruritus**, and **age** (6 years old) is classic for **chickenpox (varicella)**. Ruptured vesicles leave erosions that heal without scarring. Bullous pemphigoid typically affects elderly patients with large bullae. Pemphigus vulgaris presents with **flaccid** bullae and Nikolsky sign. Bullous impetigo presents with larger bullae and honey-colored crusts.*

Q: A 70-year-old man with a history of chronic venous insufficiency presents with a painful, shallow skin defect over the medial malleolus. Examination reveals full-thickness loss of the epidermis and upper dermis with visible granulation tissue at the base. Which of the following best describes this lesion?

- A. Erosion
- B. Ulcer
- C. Fissure
- D. Excoriations
- E. Crust

Answer: B

*Explanation: The lesion demonstrates **full-thickness loss of the epidermis and upper dermis with granulation tissue** — this is the definition of an **ulcer**. The location (medial malleolus) and history of venous insufficiency are consistent with a **venous stasis ulcer**. Erosions involve only partial epidermal loss and heal without scarring. Fissures are linear cracks. Excoriations are from scratching. Crust is dried exudate on the surface.*

Q: A 55-year-old woman presents with well-demarcated, violaceous, polygonal, pruritic papules on her wrists. Close examination reveals a fine, lacy white network of lines on the surface of the papules. This white lacy pattern is known as:

- A. Auspitz sign
- B. Nikolsky sign
- C. Wickham striae
- D. Koebner phenomenon
- E. Darier sign

Answer: C

*Explanation: The clinical description is classic for **lichen planus** — purple (violaceous), polygonal, pruritic papules on the wrists. The fine, lacy white lines on the surface are called **Wickham striae**, which are pathognomonic of lichen planus. Auspitz sign (pinpoint bleeding on removal of scale) is seen in psoriasis. Nikolsky sign (epidermal detachment with lateral pressure) is seen in pemphigus. Koebner phenomenon (lesions at sites of trauma) is seen in psoriasis and lichen planus but is described differently. Darier sign (urtication on rubbing a lesion) is seen in mastocytosis.*

Q: A patient presents with widespread skin thickening, accentuated skin markings, and leathery texture on the extensor surfaces of the elbows and knees bilaterally. The most likely diagnosis is:

- A. Lichen planus
- B. Lichenification secondary to chronic eczema
- C. Scleroderma
- D. Lichen sclerosus
- E. Morphea

Answer: B

*Explanation: The description of **thickened skin with accentuated skin markings** on extensor surfaces of elbows and knees in a pruritic context is characteristic of **lichenification**, a secondary skin lesion resulting from chronic scratching and rubbing, most commonly seen in **chronic eczema (atopic dermatitis)**. Lichen planus presents with purple, polygonal papules — not diffuse thickening. Scleroderma involves sclerosis. Lichen sclerosus causes white, atrophic plaques. Morphea causes localized sclerotic plaques.*

Q: A 45-year-old woman presents with thin, fragile skin on her arms with visible dilated blood vessels, easy bruising, and purple stretch marks. She has been using topical high-potency corticosteroids for eczema for the past 6 months. Which type of skin lesion is most consistent with her presentation?

- A. Lichenification
- B. Sclerosis
- C. Atrophy
- D. Desquamation
- E. Ulceration

Answer: C

*Explanation: The features of **thin, fragile skin, visible blood vessels (telangiectasia), easy bruising, and striae (stretch marks)** are classic for **corticosteroid-induced skin atrophy**. Atrophy is defined as loss of tissue — in this case dermal atrophy from prolonged topical steroid use. Lichenification produces thickened skin. Sclerosis produces hardened skin. Desquamation is peeling of scales. Ulceration is full-thickness tissue loss.*